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Opportunities and Challenges for the Federally Qualified Health Center Program: A Critical Review

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Abstract

Federally Qualified Health Centers (FQHCs) were founded to improve access to primary health care for medically underserved areas and populations. Since their inception in the 1960s, FQHCs have grown to provide comprehensive primary care services to more than 31 million patients in 2023. In this review, we provide an overview of the role and growth of the FQHC program, summarize the existing evidence on its association with key outcomes, and discuss opportunities and challenges for the FQHC model. Evidence finds that FQHCs have played a crucial role in expanding access to primary care for low-income populations and that quality of care delivered in FQHCs often compares similarly or favorably to that of other settings.



The ability to sustain and expand FQHCs' role is contingent, however, on stabilizing revenue streams and financing, developing comprehensive models of safety-net health care that include linkages with specialists, and implementing policies to support workforce recruitment and retention.

INTRODUCTION

Overview of the FQHC Program

Federally Qualified Health Centers (FQHCs) aim to improve access to health care for medically underserved areas and populations and are a cornerstone of the primary care safety net. In 2023, about 1,500 FQHC grantees provided care at more than 16,000 delivery sites for ~31.3 million patients (**Figure 1**) (76, 84). Nearly 90% of patients served by these FQHCs had incomes at or below 200% of the federal poverty level, 18% were uninsured, almost two-thirds were of non-White race or ethnicity, about 30% resided in rural areas, and more than one-quarter were best served in a language other than English (54, 84).

FQHCs are health systems that qualify for federal Health Center Program funding authorized in section 330 of the Public Health Service Act (PHSA) (also known as section 330 funding). FQHCs must be nonprofit or public entities that serve federally designated medically underserved areas or populations, provide comprehensive primary care and other enabling services (e.g., transportation, translation services, case management), have governing boards with a majority of members being patients of the health center, provide services to everyone in the service area regardless of a patient's ability to pay, and offer a sliding scale fee schedule by income level.

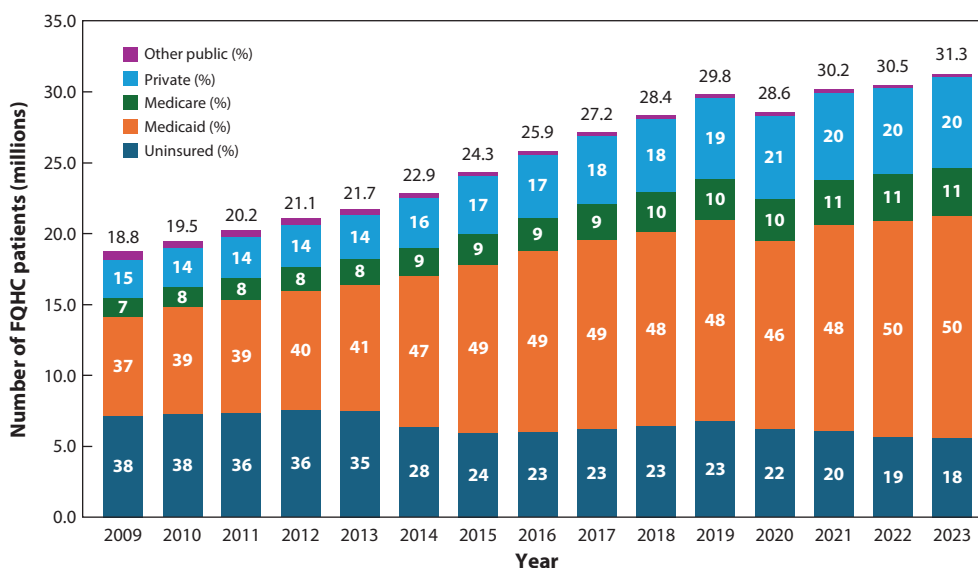


Figure 1

Total number of Federally Qualified Health Center (FQHC) patients and insurance coverage mix, 2009–2023. This figure displays the total number of patients served by FQHCs in each year by principal insurance type: uninsured, Medicaid, Medicare, private, or other public. Labels at the top of each column denote the total number of FQHC patients in millions. Labels within each column denote the percentage of FQHC patients by insurance type. Percentages may not sum to 100% because the label for Other Public is not displayed or due to rounding. All data were obtained from the Uniform Data System.

FQHCs have their roots in community health centers founded in the 1960s to respond to disparities in care in segregated and underserved communities (28). Drs. Aaron Shirley, James Anderson, Robert Smith, and other physicians practicing in Mississippi, in collaboration with northern physicians and public health advocates Drs. H. Jack Geiger, Count Gibson, and John W. Hatch, developed and piloted the community health center model (28, 107). The first neighborhood health centers were established in rural Mississippi and urban Boston under the Economic Opportunity Act of 1964, which was part of the broader War on Poverty under President Lyndon B. Johnson (105). The goals of these community health centers were not only to provide direct medical care, but also to empower communities, spur economic development, and address root causes of health inequities (37, 47).

Following the initial 10-year demonstration program, the Community Health Center program was permanently authorized in 1975 by the US Congress with the enactment of section 330 of the PHSA. The PHSA also outlined specific requirements, including service to medically underserved areas (42 U.S.C. § 254b), for health centers to be eligible for federal grants. The medically underserved designation by the Health Resources and Services Administration identifies areas or populations with limited access to primary care services based on an index that includes four criteria: primary care provider to population ratios, poverty levels, infant mortality rates, and the proportion of the population aged 65 years old or older (68). The Omnibus Reconciliation Act of 1989 established the FQHC designation to allow qualified community health centers to receive cost-based reimbursements from Medicaid and Medicare, improving the centers' financial self-sustainability as these rates are typically higher than what FQHCs would receive otherwise (34). The Health Center Consolidation Act of 1996 combined federal section 330 funding and special population funding authorities into a consolidated program that includes general FQHCs, Migrant Health Centers, Health Care for the Homeless programs, and Public Housing Primary Care programs. The FQHC program also includes FQHC look-alikes that meet the requirements for section 330 funding, but do not receive it (134 look-alike centers served 1.2 million patients in 2023) (77).

In addition to being eligible for section 330 funding, FQHCs qualify for malpractice coverage through the Federal Tort Claims Act and obtain federal loan guarantees for capital improvements (12). FQHCs are eligible for other sources of federal support, such as the National Health Service Corps, which provides scholarships and loan repayment for primary care providers in medically underserved areas, and the 340B Drug Pricing Program, which requires drug manufacturers to provide prescription drugs to safety-net providers, including FQHCs, at discounted prices.

While the focus of FQHCs is on primary care, a growing number have expanded service offerings to include dental care, on-site pharmacies, mental health and substance use disorder services, and, to a lesser extent, vision care (75). There has been notable growth in the availability of on-site mental health and substance use disorder care at FQHCs over the last decade, reflecting broader efforts and support for integrating behavioral and primary health care and growing demand for this care (11). In a national survey of FQHC systems in 2024, 88% reported that their largest site usually or often offered on-site short-term mental health counseling, 70% provided long-term mental health counseling, 66% offered substance use disorder treatment, and 62% provided medication-assisted substance use disorder treatment (52). Between 2018 and 2022, the number of behavioral health visits at FQHCs increased by 39%, and behavioral health conditions are now the most common diagnoses associated with FQHC visits (50).

Expansion of the FQHC Program

FQHCs have often garnered bipartisan support, spurring their growth, though with potentially varying policy objectives. Conservative policymakers have tended to view FQHCs as a means

of providing direct care to underserved populations and an alternative to expanding the federal government's role in subsidizing insurance coverage. In contrast, progressive policymakers tend to view FQHCs as complementary to expansions in insurance coverage and an effective model for addressing adverse social determinants of health (SDOH) and reducing health inequities. For example, President George W. Bush's Health Center Growth Initiative, launched in 2001, added more than 1,200 FQHC delivery sites and increased the number of patients served by 60% to 16.25 million by 2007 (82, 90). By strengthening the safety net, the initiative may have helped temper pressure for broader federal Medicaid expansions.

By comparison, the Affordable Care Act (ACA) under President Barak Obama allocated \$1.5 billion for the Health Center Program and \$9.5 billion for the Community Health Center Fund (CHCF) between 2011 and 2015, in part to expand primary care availability for low-income populations expected to gain coverage through Medicaid expansion and the ACA private insurance marketplaces. Between 2010 and 2015, the number of FQHC grantees increased from 1,124 to 1,375, the number of FQHC sites grew from 6,949 to 9,754 (FQHC grantees often have multiple delivery sites), and the total number of patients served grew from 19.5 million to 24.3 million (Figure 1) (71). With ACA-related expansions in health insurance coverage, the insurance mix of FQHC patients has also shifted substantially since the ACA's enactment. In 2009, FQHC patients were most likely to be uninsured (38%), followed by having Medicaid coverage (37%); as of 2023, those with Medicaid coverage comprised 50% of FQHC patients, and 18% of patients were uninsured (Figure 1).

Correspondingly, Medicaid reimbursement now comprises the largest share of FQHC revenue (43% in 2023), followed by private insurance reimbursement (12%) and federal section 330 funding (11%), highlighting the importance of both Medicaid policy and specific federal appropriations to the sustainability and growth of FQHCs. The remaining revenues for FQHCs stem from Medicare reimbursements; other federal, state, and private grants and contracts; and self-pay (46, 90). FQHCs in states that expanded Medicaid experienced greater decreases in the proportion of patients who are uninsured (45), and studies comparing changes in Medicaid expansion versus nonexpansion states have found that FQHCs in expansion states had greater increases in FQHC visit volumes and improved quality of care on certain metrics, such as asthma treatment and hypertension control (20, 45).

The CHCF created by the ACA now comprises most of the section 330 funding and must be continually reauthorized by Congress (48). Evidence finds positive associations between funding increases and the number of FQHC delivery sites (4), patients served (93), and FQHC service availability and offerings (64). From 2010 to 2015, an estimated 1,242 medically underserved ZIP codes were newly included in FQHC service areas, representing about 18.4 million residents who may have gained access to an FQHC (4). However, concern and uncertainty remain on the extent to which such FQHC expansions have targeted geographic areas with the greatest needs, such as rural areas and those with higher levels of poverty (4, 16, 31).

FQHC Care Delivery in Rural Areas

FQHCs play a critical role in delivering primary care in rural regions, which are more commonly located in the US Southeast and Midwest. In 2023, about 41% of FQHCs were in rural areas, operating 6,497 delivery sites and serving about 10 million patients, representing about 20% of rural residents (14, 74, 110). One study found that rural patients seen at FQHCs were more likely to receive certain preventive services, such as Pap smears and pneumococcal vaccinations, than the general rural population (29). Moreover, evidence suggests that FQHCs play an important role in maintaining access to medical services in rural communities experiencing hospital closures, such as

by opening new FQHC delivery sites or collaborating with distressed hospitals to acquire buildings and facilities or assume certain services, most commonly primary, prenatal, and behavioral health services (14).

Rural FQHCs also face unique challenges in delivering care and addressing health disparities. Compared with urban patients, for example, rural populations tend to be older and to have higher rates of chronic disease (36). Moreover, rural FQHCs face substantial difficulties recruiting and retaining providers—including primary care physicians, nurses, dentists, and behavioral health care specialists—and provider shortages in rural areas also impact FQHCs' ability to link patients with specialty care (80). While the availability of telehealth has increased dramatically across all FQHCs since the COVID-19 pandemic, the lack of broadband access in some rural areas limits patient access to telehealth services, and rural FQHCs are more likely than urban FQHCs to report facing financial barriers to offering telehealth (13).

REVIEW OF THE EVIDENCE ON FQHCs: ACCESS, QUALITY, AND SOCIAL DETERMINANTS OF HEALTH

Access to and Use of Care

Expanding the number of FQHC sites could have important implications for access to care and utilization patterns for underserved populations. FQHCs are more likely to offer off-hour visits (e.g., evening and weekend) compared with non-FQHC primary care providers (94). Audit studies have found that new patient appointment rates for Medicaid beneficiaries are 13.5–22.0 percentage points higher in FQHCs compared to other office-based providers, but wait times are similar after receiving an appointment (88, 91). Studies have also found associations between individual-level geographic proximity to FQHCs, such as living in a county with an FQHC or with a shorter distance to an FQHC, and improvements in patient-reported access to care measures, such as having a usual source of care and decreases in delayed or avoided care due to cost (10, 43, 44).

Improved access to FQHCs is associated with increases in the likelihood of having physician visits and ambulatory service use (44, 67). Highlighting the relationship between FQHC availability and insurance coverage on access to care, McMorro & Zuckerman (67) found that living in health care markets that had higher funding during the Health Center Growth Initiative from 2001 to 2008 mitigated declines in some but not all measures of access, including having a usual source of care and any office visit, for low-income uninsured and publicly insured individuals but not for those with private coverage. Furthermore, greater FQHC funding growth was associated with a 4% reduction in unmet dental care needs for low-income privately insured adults, suggesting the role of FQHCs in providing access to oral health care that is often not included in private coverage (67).

Increases in primary care availability through FQHCs could shift certain types of care from emergency departments (EDs) to more cost-effective outpatient settings, particularly for low-income patients who more often lack usual sources of care. Cross-sectional studies comparing ED use for FQHC versus non-FQHC patients have mixed findings; however, some of these studies are restricted to specific states or patient subgroups, and cross-sectional studies are likely to be subject to bias due to systematic differences in characteristics of patients served by FQHCs versus other settings (18, 61, 85, 89). Meanwhile, longitudinal studies examining increases in FQHC availability and changes in ED use have found more consistent, inverse associations, suggesting that FQHCs help to reduce costly ED visits, particularly for nonemergent conditions that can be treated in office-based settings (67, 69, 79).



Access to Specialty Care

In contrast to primary care, numerous studies have documented limited access to specialty care for FQHC patients (23, 92, 94, 103). Many studies have focused on single sites or a subset of FQHCs, and rates of incomplete referrals range from about 15% to 30%, with wide variation across specialties (113). In a two-site study focused on children, 42% of families encountered barriers to completing specialty referrals, with commonly cited barriers including difficulty taking time off work, obtaining childcare, and finding transportation (114). Shortages of specialists, particularly those who are willing to accept individuals with Medicaid or no insurance or who are included in managed care provider networks, also present barriers to care for many FQHC patients (32). In a 2024 national survey, most FQHCs reported that it was difficult for patients to obtain timely specialty care appointments, with the greatest difficulty for patients with Medicaid or without insurance (73%) compared with Medicare patients (54%) and insured patients (41%) (52). Those who are uninsured are likely to face the greatest difficulty completing specialty referrals (23, 92). However, whether receipt of specialty care is more limited for uninsured individuals or Medicaid beneficiaries who receive care at FQHCs compared with non-FQHC settings has not been established, and the difficulties related to obtaining timely specialty care identified for FQHC patients have been found to apply more broadly to low-income patients (6, 35, 100).

Chronic Disease Care and Health Care Quality

Older studies examining the quality of primary care for patients receiving care in FQHCs have generally found that while quality gaps persist, FQHCs perform similarly to and, in some cases, more favorably compared to other primary care settings on quality measures, particularly related to preventive care (39, 49, 59, 95, 96, 98). For example, using data from national surveys in 2002, Shi & Stevens (96) found that uninsured and Medicaid FQHC patients were more likely to receive certain cancer screenings (e.g., Pap test, mammogram) and preventive counseling (e.g., smoking, exercise) compared with similar patients nationally. FQHCs have also performed similarly to private practices with respect to the delivery of high-value (e.g., prescribing of beta blockers for congestive heart failure) and low-value (e.g., screening electrocardiograms, antibiotic prescription for upper respiratory infection) services (83).

Findings related to chronic disease care quality measures are more mixed. For example, Medicaid-insured patients with diabetes seen at FQHCs versus non-FQHC settings have been found to be about 5–8% less likely to get recommended laboratory tests [e.g., hemoglobin A1c (HbA1c), cholesterol] and retinal exams, but also more likely to receive diabetes education and less likely to have diabetes-related hospitalizations (59). Another study found that FQHCs tended to outperform Medicaid managed care organizations on diabetes control (98). In a study comparing Medicaid-insured individuals with serious mental illness, those receiving care primarily in FQHCs versus other settings were 2% more likely to have outpatient mental health visits and 4% more likely to have guideline-consistent psychotropic medication fills (70).

Studies using ecological data have also examined whether access to or use of FQHCs is associated with rates of potentially preventable hospitalizations and ED visits and found lower rates of these adverse events among those living in areas with greater access to FQHCs (30, 87). Some cross-sectional studies at the patient level have attempted to account for differences in the characteristics of FQHC versus non-FQHC patients by limiting comparisons to patients with similar insurance status and/or those living in similar areas and controlling for other potential confounders. Findings from these studies are more mixed. For example, among Medicaid beneficiaries in four states, patients seen primarily in FQHCs versus other settings had 11% and 19% lower odds of ambulatory-care sensitive condition (ACSC) ED visits and hospitalizations (33).

However, among dual-eligible Medicare–Medicaid beneficiaries, relationships varied by race, ethnicity, and disability status (111). For example, among disabled dual-eligibles under age 65, Black and Hispanic FQHC patients had slightly higher rates of ACSC ED visits and hospitalizations, but White FQHC patients had lower rates compared with similar patients without FQHC use (85).

Stronger evidence on the effects of FQHCs on quality and outcomes remains lacking due to the methodological challenges of fully accounting for differences in the characteristics of patients seen at FQHCs versus other care settings. A recent analysis of a natural experiment examining FQHC closures found that clinic site losses were associated with increases in county-level mortality (3.54 per 100,000 population) and that this effect was mediated by decreasing primary care provider density and patient volume (3). Additional studies leveraging strong quasi-experimental designs to assess the causal effects of FQHCs on patient outcomes are needed.

Enabling Services and Social Determinants of Health

Responding to a holistic set of factors that contribute to health disparities for poor and marginalized populations has been part of the FQHC program's mission and model since its inception. FQHCs are required to offer nonclinical enabling services that promote health care access, such as care coordination, language interpretation, and transportation services. A study estimated that 80% of FQHC patients in 2014 received at least one enabling service (most commonly, care coordination, health education, and free medication) and that similar patients who received versus did not receive enabling services had an average of 1.92 more health center visits, a 16-percentage-point higher likelihood of having a recent flu shot, and an eight-percentage-point higher likelihood of patient satisfaction (112). Disentangling the effect of any individual service is challenging, however, given the interrelated nature of needs for enabling services.

Beyond enabling services, FQHCs' long-standing relationships within underserved communities can enhance patient trust, reduce stigma, and also facilitate discussions about the SDOH (26, 42). For example, in 2022, 22% of patients reported receiving economic-related assistance from their FQHC, most commonly receiving help applying for government programs such as Medicaid and nutrition assistance (e.g., through the Supplemental Nutrition Assistance Program) or connecting with community-based food or housing services (84). FQHC SDOH services are provided both on-site and through partnerships with community-based organizations. As such, the majority of FQHCs (70% in 2020) routinely collect data on patients' social risk factors (21, 97). Sixty percent of these FQHCs reported using at least one standardized screening tool, with the most common being the PRAPARE (Protocol for Responding to and Assessing Patients' Assets, Risks, and Experience) screener, although variability in the screeners used across FQHCs has been identified as a barrier to sharing SDOH data (24).

While screening for social needs is an important first step in addressing SDOH, FQHCs nevertheless face challenges in incorporating this information into clinical workflows and connecting patients with available and effective services in the community (62, 86). The challenges identified for FQHCs echo challenges identified more broadly across health care organizations attempting to address patients' social needs. For example, some FQHCs that screen for patients' health-related social needs do not have systematic approaches for sharing these data with care teams or discussing them with patients (86). Limited availability of services and resources—such as affordable housing—also limits opportunities to address social risk factors. In addition, bidirectional data exchange between FQHCs and community-based organizations is complicated by the use of different data collection systems and platforms and Health Insurance Portability and Accountability Act (HIPAA)-related concerns about the sharing of patient information (108). The lack of

standardized screening tools and data-sharing systems across health care and social service sectors further complicates efforts to track patient outcomes and coordinate care effectively, highlighting the need to better systematize these processes.

CHALLENGES AND OPPORTUNITIES FOR THE FQHC MODEL OF CARE

Despite the body of evidence highlighting the role of FQHCs in improving access to and quality of care for medically underserved populations, FQHCs face numerous challenges regarding their financial stability, linkages with specialty care, and workforce recruitment and retention. FQHCs operate on thin margins and rely heavily on federal and state grants, as well as Medicaid reimbursements, to provide care for medically underserved populations. However, fluctuations and uncertainty in these financial streams pose significant challenges and affect the ability of FQHCs to sustain operations, expand services, and invest in necessary workforce and infrastructure improvements.

Uncertainty of Federal Funding

FQHCs receive federal section 330 funding through both discretionary funding and mandatory funding through the CHCF. A recent analysis found that total federal funding for FQHCs, which increased from \$5.0 billion in 2015 to \$5.6 billion in 2022, decreased by 12% in real terms, accounting for medical care inflation, and represents a decrease of 30% in per-patient funding (9). In addition, delayed or short-term reauthorizations of the CHCF since its original five-year authorization ended in 2015 have led to considerable financial uncertainty for FQHCs and adversely impacted their ability to maintain staffing and service provision (56, 57). When CHCF funding lapsed for 5 months in 2017, 20% of health centers reported instituting a hiring freeze, 25% canceled or delayed planned facility renovation or expansion, 14% canceled or delayed quality improvement or infrastructure investment, and 11% tapped into reserve funds (56). Sustained and long-term funding is needed to reduce financial unpredictability for FQHCs and enable longer-term strategic planning.

FQHC leaders have also noted that recent challenges to the 340B prescription drug program, including reduced participation by some pharmaceutical manufacturers, have led to budget concerns for their health centers (51). The 340B program allows safety-net providers to purchase prescription drugs at reduced prices; these savings should then be used by providers to expand access to care for patients. However, growth in the number and types of organizations that participate in the 340B program and concerns about a lack of transparency have led to calls for reform and scalebacks of the program by policymakers and drug manufacturers (19). Broad scalebacks, however, could have major implications for FQHC patients. A recent study found that greater 340B engagement by FQHCs was associated with increases in activities consistent with the provision of safety-net care, including increased patient volumes among uninsured and unhoused patients and increases in visits providing low-profit/high-value preventive services, including HIV tests, tobacco cessation counseling, and influenza vaccinations (109). Moreover, nearly 90% of FQHCs participating in a National Association of Community Health Center (NACHC) survey about the 340B program noted that the program has led to improved access to care, medication adherence, and health outcomes for their patients (72).

Variability in Medicaid Revenue

Medicaid reimbursements for patient care are the largest source of revenue for FQHCs. Broad Medicaid funding cuts and reductions in Medicaid enrollment would reduce critically needed

Medicaid revenues and increase the amount of uncompensated care provided by FQHCs. Congressional Budget Office projections from June 2025 estimate that the number of uninsured individuals in the United States will grow by 16.0 million by 2034, with about half of the losses due to reductions in Medicaid coverage from provisions in the One Big Beautiful Bill Act of 2025 (102). These provisions include requirements for states to establish work-related requirements for coverage and to conduct eligibility redeterminations every six months instead of annually for certain Medicaid enrollees (102). Other estimates suggest that more than 5.5 million FQHC patients with Medicaid could lose coverage over the immediate 5 years after work requirements are implemented, with corresponding losses in revenue for FQHCs of almost \$32 billion. Unless these policies are paired with additional FQHC funding from federal and state governments, FQHC capacity could decrease drastically during a time of rapid growth in the uninsured population and demand for safety-net care (60).

Insufficient reimbursement, administrative burdens, and delays associated with Medicaid billing can also strain FQHC operations and divert resources away from direct patient care. The Medicare, Medicaid, and SCHIP Benefits Improvement and Protection Act of 2000 created a prospective payment system (PPS) that Medicaid is required to use to reimburse FQHCs using a per-visit payment rate (65). Each FQHC has a specific PPS rate based on its allowable costs that are adjusted for inflation and are intended to reflect the scope of services furnished by FQHCs. Many FQHCs report concerns that PPS rates are insufficient and outdated. The NACHC estimates that current PPS rates cover only about 82% of an FQHC's cost of caring for a Medicaid patient, on average (101). Some critics also note that since its origin, the PPS rate has not kept pace with changes in care delivery and does not adequately account for patient complexity. There is also wide variation in PPS rates across states, with some states not having adjusted their rates for many years. Some states have policies that may limit FQHCs' ability to obtain PPS rate adjustments, such as requiring a 3% minimum change in cost or recognizing only scope changes associated with new services but not changes associated with increases in intensity, duration, or amount of services provided (41, 53). Moreover, a 2018 report found that only 36 states had a documented definition of a "change in the scope of services" and that some of these states had incomplete documentation (40). Such ambiguities can create challenges for FQHCs, particularly when they adopt or discontinue service types, add new clinical technology, or change their provider mix, and can potentially discourage FQHCs from making such changes altogether.

Payments from Medicaid managed care organizations (MCOs) can also introduce complexity and delays in reimbursement. In 2022, 75% of all Medicaid enrollees were in a comprehensive MCO with wide variation across states and eligibility groups (58). Medicaid MCOs have more flexibility in how they pay FQHCs, although state Medicaid programs are required to provide wraparound payments to FQHCs if MCO payments are less than what FQHCs would have been paid under the PPS rate (65). Yet FQHCs report that they often do not receive correct or timely wraparound payments (41).

Alternative and Value-Based Payment Models

States may also choose to use an alternative payment methodology (APM) instead of the PPS rate if payments are equal to or greater than what an FQHC would receive under the PPS. States use a variety of APMs, with some states experimenting with bundled payments, per-member per-month payments, and value-based payment (VBP) models that incentivize FQHCs to provide high-value care, such as through quality bonuses, shared savings, or risk models (41). Growth in the use of VBP presents opportunities to address shortcomings in current payment methods and spur innovation at FQHCs in both clinical and SDOH-related care. However, FQHC participation



remains somewhat limited; a recent report estimated that while 40% of FQHCs participate in some form of VBP, these payments account for less than 5% of revenue (17).

Interviews with FQHC leaders highlight that many view VBP models favorably and feel that FQHCs already have experience with several activities that promote value-based care, such as care coordination and integrated care models (63). Nevertheless, FQHCs also face barriers to participating in VBP models, including the need to establish the necessary infrastructure to enter into VBP arrangements; such infrastructure includes administrative and technical resources for managing multiple contracts and information technology capabilities to capture and report quality measures (38, 63). Moreover, while FQHCs are already required to report on quality measures as a requirement of receiving federal funding, VBP contracts often use quality measures that are constructed differently from the measures FQHCs already report, and quality measures can also vary across VBP contracts, contributing to reporting and informatics burdens for FQHCs (38). Indeed, a practical challenge to maximizing the benefits of VBP contracts for FQHCs is the need to manage multiple payment models. Having multiple payment models that are not aligned in reimbursement methodology adds administrative complexity, which can undermine the advantages VBP contracts offer.

FQHC leaders have also expressed concern that risk adjustment models must adequately account for the clinical and social complexity of FQHC patient populations to ensure that FQHCs are not unfairly penalized for caring for high-risk patients (63). Limited specialty care networks, particularly for Medicaid and uninsured patients, also raise concerns for FQHCs about entering contracts that carry greater financial risk. Providing upfront financial and operational support for FQHCs to build the necessary infrastructure, aligning quality measures with existing reporting requirements, facilitating linkages with specialists, and allowing FQHCs to enter into VBPs in a stepwise fashion may help facilitate successful transitions into VBP arrangements (17, 38, 104).

Linkages with Specialty Care

Challenges that FQHC patients have experienced in accessing specialty care have been attributed to a range of barriers commonly faced by low-income populations, particularly those who are uninsured (7, 27, 99, 100). Although Medicaid coverage has expanded among the FQHC patient populations, FQHC directors have reported that low Medicaid payment rates, limited provider networks in Medicaid MCOs, and MCO administrative requirements contribute to challenges in finding specialty care for these patients (103). FQHCs seeking to provide additional specialty care on-site face complex legal and regulatory requirements that have been historically difficult to comply with, including the need to make services offered by FQHCs available to all patients equally, regardless of insurance status or ability to pay (8). These challenges are compounded in rural areas, where specialty providers are more scarce (25).

A recent scoping review identified commonly used strategies that FQHCs are employing to address barriers to specialty care, including the use of virtual consults, expansion of the role of nonspecialist providers, greater use of specialty referral coordinators, strengthening of the relationships and contractual agreements with specialist institutions, integration of specialty care into on-site services, and the use of algorithms to help prioritize high-need patients (78). For example, the use of an eConsult platform at a large FQHC that connected primary care providers with specialists in dermatology, endocrinology, gastroenterology, and orthopedics reduced the need for face-to-face visits, allowing many patients to be managed in primary care (2). A case study of a recent partnership between an FQHC and academic medical center in Baltimore noted the advantages of combining FQHCs' strengths in primary care delivery in medically underserved communities with access to extensive specialty networks available at academic centers (22). The

use of various strategies differs across FQHCs depending on local conditions, including patient needs and the local availability of specialists (106). Additional research is needed to identify effective strategies for improving the receipt of timely specialty care linked with context and to document best practices for implementing strategies.

Workforce

Workforce shortages also pose significant challenges for FQHCs in meeting the demand for care, and a growing number of FQHCs have reported shortages since 2018. For example, the share of FQHCs reporting a shortage of mental health providers grew from 70% to 77% and of primary care physicians from 65% to 70% from 2018 to 2024. Even larger increases were observed for nurses and enabling services providers (e.g., social workers, community health workers), which both grew from 54% to 70% and 69%, respectively (52). The strain on the FQHC workforce was exacerbated by the COVID-19 pandemic. An NACHC survey conducted in March 2022 found that 68% of FQHCs reported losing between 5% and 25% of their staff, and 15% of FQHCs reported losing between 25% and 50% of their staff over the previous 6 months. Pandemic-related stress and better financial opportunities at larger health care organizations were the most common reasons for departure (73).

Potential restrictions related to immigration policy and visas for international medical graduates (IMGs) could also have dire consequences for the FQHC workforce. An estimated 25% of licensed physicians in the United States are IMGs (1), and these physicians are disproportionately represented in primary care, rural areas, and underserved urban areas (55). IMGs who commit to working for a set period in Health Professional Shortage Areas or medically underserved areas may qualify for certain visa waivers, such as J-1 visa waivers or National Interest Waivers, to stay in the United States. Some evidence suggests that many IMGs continue to work in medically underserved areas beyond their service requirement (5). Nevertheless, many IMGs face challenges to staying in the United States due to limits on visas and difficulties obtaining green cards (66), and some FQHCs report that burdensome applications are a barrier to participating in the J-1 visa waiver program (81).

The NACHC has recommended numerous policies that would help FQHCs address workforce recruitment and retention, including continued federal support for workforce programs such as the National Health Service Corps, the Teaching Health Center Graduate Medical Education program, and the Nurse Corps Scholarship Program. Moreover, expanding the types of billable providers, such as peer support workers and community health workers, who may participate in integrated care teams and expanding the scope of practice laws for advanced practice providers, such as nurse practitioners and dental therapists, could also help to alleviate workforce barriers (73). Such policies could be particularly valuable for rural FQHCs that face greater challenges than urban FQHCs with recruiting and retaining physician providers and that have a comparatively larger ratio of nonphysician to physician providers (15). To complement such efforts, evidence on effective models for reducing provider burnout and for team-based care is needed.

CONCLUSIONS AND FUTURE DIRECTIONS

Community health centers have evolved over the last 60 years to become essential providers of comprehensive, community-based care for more than 30 million patients across the United States. Their impact on improving access to primary care, particularly for low-income, uninsured, and racially and ethnically diverse populations, is well-documented. FQHCs have also demonstrated a growing capacity to deliver behavioral health care and address SDOH. Yet their ability to



sustain and expand this role remains contingent on stable financing, sustainable payment models, improved linkages with specialty care, and a robust workforce.

FQHCs face a particularly challenging policy environment following the passage of the 2025 Federal Budget Reconciliation Bill, which is projected to substantially reduce Medicaid coverage and increase the number of uninsured individuals. Future policy efforts to align funding streams and workforce investments with the realities of FQHC service delivery, patient needs, and demand for safety-net care will be critical. Providing upfront support for FQHCs to participate in VBP models could help address concerns related to outdated and insufficient payment models. Research is needed to build the evidence base on the most effective approaches to care delivery, particularly in relation to access to specialty care, team-based care models, workforce recruitment and retention, and cross-sector collaborations to address social risks. FQHCs are uniquely positioned to contribute to advances in value-based care and the promotion of health equity. Ensuring that policies and resources are aligned to promote their ability to do so is a critical step toward a more accessible and equitable health care system.

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